NCDI POVERTY NETWORK

Version 2.0. April 2026

PEN-Plus Mentorship Tool

Mentee name:

Conditions seen at PEN-Plus Clinic:

Site Mentoring Activity: ☐Daily ☐Weekly ☐Monthly ☐Occasionally ☐Unscheduled

Scoring system: Goal level 4 or 5

	1	2	3	4	5	N/A or 0
Mentee	Does not demonstrate competency	Demonstrates basic competency	Demonstrates satisfactory competency	Demonstrates advanced competency	Demonstrates exceptional competency	The competency cannot be evaluated
Mentor	I had to do	I needed to talk them through	I needed to prompt	I needed to be there just in case	I did not need to be there	

The main objective of the mentorship tool is to assess clinical practice, measure clinical autonomy, and identify specific GAPs to develop specific teaching activities.

GREY COLOR MEANS: ADVANCED COMPETENCIES. They are not required to be achieved to be considered a competent provider.

PHASES OF MENTORSHIP in the **Ideal package of training** *(For additional options, please refer to the PEN-Plus training framework document outlining the basic and intermediate training packages)*

1. INITIAL INTENSIVE MENTORSHIP:
    a. Frequent direct observation and mentorship. 4 weeks continued or 1 week every month (the duration could change based on the achievement of the level of competency)
    b. **Goal to next phase when**: Overall autonomy ≥ 3 on repeated observations (at least 70% in each disease of the basic competencies evaluated)
2. ON-GOING MENTORSHIP
    a. Less frequent observation and mentorship with dedicated observational support and more case discussion. Between 3-6 months monthly.
    b. **Goal to exit phase:** consistent domain scores ≥ 4 with safe independent case handling (at least 70% in each disease of the basic competencies evaluated)
3. SUPERVISION MODEL (oversight rather than teaching)
    a. Random audits on complex cases, quality assurance, or system strengthening.

DIABETES MELLITUS

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
	PATIENT ASSESSMENT AND CLASSIFICATION					
D1	Performs a comprehensive initial evaluation of a patient with diabetes including history, physical exam, identification of complications and baseline investigations					
D2	Identifies what patients are type 1, type 2 and other types based on clinical presentation and revises the diagnosis when new information emerges					
D3	Measures blood glucose at each visit and interprets pre- and post-prandial levels of glucose according to recommended targets.					
	TREATMENT ADJUSTMENT AND INDIVIDUALIZATION type 1 DM					
D4	Calculates the initial total daily dose of insulin using available insulin regimens					
D5	Assesses glycemic control using glucose logbooks, clinical history, available laboratory results (Hb1Ac) and adjust treatment safely and adequately					
D6	Safely titrates insulin doses for food insecurity					
D7	Safely titrates insulin doses for sick day					
D8	Safely titrates insulin for Ramadan					
D9	Safely titrates insulin doses for physical activity					
D10	Safely transitions patients between insulin regimens when indicated					
D11	Initiates the use of correction scales and teaches them appropriately					
	TREATMENT ADJUSTMENT AND INDIVIDUALIZATION type 2 DM					
D12	Initiates metformin correctly including correct dosing and contraindications					
D13	Titrates correctly metformin to effective dose					
D14	Adds second line therapy (sulphonyl urea) when glycemic targets are not achieved					
D15	Starts insulin therapy when glycemic targets are not achieved					
D16	Titrates insulin therapy in type 2 if uncontrolled glucose targets or Hb1Ac.					

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
	SAFETY					
D17	Diagnoses and initiates correct management or referral for symptomatic hypoglycemia					
D18	Prioritizes prevention and correction of hypoglycemia when adjusting diabetes treatment					
D19	Understands and orders ketone testing when indicated					
D20	Identifies hyperglycemic emergencies and initiates first line stabilization or referral					
D21	Understands indications for hospitalization in hyperglycemic status and sends for hospitalization					
	SCREENING and INITIAL MANAGEMENT FOR CHRONIC COMPLICATIONS					
D22	Screens for feet complication in each visit and identifies patients at risk of foot complications					
D23	Assesses eye and refers for evaluation in the first visit and every year					
D24	Assesses kidney damage ordering urine proteins and kidney function (if available) in the first visit and every year					
D25	Assesses sexual dysfunction and refers to the urologist if it happens.					
D26	Starts ACE inhibitors if BP > 140/80 and/or evidence of kidney damage					
D27	Measures creatinine in patients with ACE inhibitors after starting or increasing the dose within the next 4 weeks. Holding if there is an increase of more than 30% from the initial creatinine levels.					
D28	Adds statins to all patients with Diabetes Mellitus over 45 years old					
	PATIENT EDUCATION					
D29	The provider teaches a patient how to use a home glucometer and document blood glucose into logbook, blood sugar ranges and treatment goals.					
D30	The provider teaches insulin administration and injection techniques, and insulin storage.					
D31	The provider teaches on correction scales when the patient if appropriate for the patient					
D32	The provider teaches nutrition and diet principles.					
D33	The provider teaches sick day management and DKA warning signs.					
D34	The provider teaches about how to adjust insulin with physical activity					
	SPECIAL SITUATION: PREGNANCY					

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
D35	The provider integrates care with ANC services or Obst/gynae units					
D36	The provider programs frequent monitoring (every 2-4 weeks until week 30 and every 2 weeks until 38 weeks) and adjusts insulin regimen based on glucose logbook					
D37	The provider starts low dose aspirin (75-150 mg) in week 12					

PATIENT EDUCATION AND SELF MANAGEMENT (if applicable)

	General Counselling Competencies (SME)	Date 1	Date 2	Date 3	Date 4	Date 5
DC1	The provider asks patients about biggest concerns at the beginning of the visit					
DC2	The provider reviews learning points and goals from the previous visit.					
DC3	The provider assesses patient's knowledge from last visits before providing new concepts					
DC4	The provider uses different educational materials available at the sites (hands-out, flip charts, pictures,) to explain concepts					
DC5	The providers prioritize the specific counselling/teaching objectives, preferably using SMART criteria, that are needed in each visit.					
DC6	The provider involves the patient in decision making (changing regimens, learning goals,)					
DC7	The provider uses the teach–back or return demonstration to ensure the patient understands what has been taught.					
DC8	The provider uses empathic language avoiding judgmental language and fear tactics					
DC9	The provider finishes the conversation with encouraging comments for achieving goals, such as confidence statement					

HEART DISEASES

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
	CLINICAL ASSESSMENT HEART FAILURE					
C1	Identifies symptoms suggestive of heart failure					
C2	Recognizes signs of clinical decompensation and correctly assesses/categorizes volume status assessment					
C3	Uses history to correctly classify heart failure according to NYHA functional class					
C4	Interprets basic echocardiography findings from a report and classifies correctly the underlying condition of heart failure based on that					
	GENERAL MANAGEMENT OF HEART FAILURE					
C5	Differentiates management between cardiomyopathies with low Ejection fraction and hypertensive heart diseases (hypertensive cardiomyopathy)					
C6	Initiates heart failure mortality reducing drugs (beta blockers, ACE inhibitors) for cardiomyopathy safely based on HF protocol					
C7	Titrates heart failure mortality reducing medicines safely (beta blockers, ACE inhibitors, spironolactone) based on HF protocol					
C8	Identifies adverse effects of ACE inhibitors, betablockers and spironolactone					
C9	Initiates diuretics safely when there are signs of HF decompensation in adults and/or children					
C10	Titrates diuretics up and down based on symptoms, physical exam and renal function					
C11	Monitors renal function & potassium levels in patients with ACE inhibitors and/or spironolactone					
C12	Recognizes heart failure decompensation and common triggers (infection, anemia, arrythmia, medication, non-adherence,..) and refers appropriately for hospitalization when indicated					
C13	Consistently recognizes when a patient's condition exceeds their level of autonomy—based on severity, complexity, or treatment response—and seeks timely specialist support, providing clear clinical summaries and specific questions to guide decision-making.					

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
C14	Performs and interpret basic ECG findings relevant to heart failure and valvular diseases					
C15	Identifies signs of heart failure in an X-ray Thorax					
	RHEUMATIC HEART DISEASES					
C16	Recognizes clinical features of rheumatic heart diseases with valvular complications (murmur in the exam)					
C17	Initiates appropriate medical management including secondary prophylaxis with penicillin					
C18	Initiates beta blockers in mitral stenosis and identifies the indication for rate control					
C19	Identifies atrial fibrillation in the EKG and starts or indicates anticoagulation to prevent systemic embolic complications					
C20	Refers to Cardiologist always regardless the symptoms of the patient or the severity of stenosis					
C21	Refers women patients with cardiac diseases to family planning and consults for planning delivery					
	CONGENITAL HEART DISEASES					
C22	Identifies loud murmur on the physical exam					
C23	Looks for and correctly identifies cyanosis and clubbing in children as signs of possible signs of congenital heart diseases					
C24	Initiates appropriate initial management based on suspected diagnosis and clinical status.					
C25	Refers to pediatric cardiologist always foe definitive evaluation					
	PATIENT EDUCATION					
C26	Provides counselling related to medication dose and side effects (e.g. syncope) and supports patient adherence by explaining the purpose and importance of the medication					
C27	Educated patients and caregivers to recognize early signs of cardiac decompensation, explains common triggers of heart failure decompensation					
C28	Teaches appropriate self-management strategies when indicated: including diuretic adjustment, according to symptoms and/or ECA inhibitors and betablockers titration					
C29	Provides counselling on diet and lifestyle modifications relevant to cardiac diseases such as salt reduction, physical activity, ...					
C30	Counsels patients and families appropriately on family planning when cardiac diagnosis is present					

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
C31	Educates patients on dietary considerations when taking warfarin.					
	HEART FAILURE MANAGEMENT IN PREGNANCY					
C32	Avoids ACE inhibitors, spironolactone and atenolol in pregnant women.					
C33	Uses pregnancy saved alternatives (Hydralazine and isosorbide dinitrate) if necessary and other betablockers apart from atenolol					
C34	Uses low doses of betablockers, ACE inhibitors and spironolactone in breastfeeding.					
C35	Avoids warfarin during first and third trimester of pregnancy if anticoagulation is indicated.					
C36	Coordinates care with cardiology and antenatal services for pregnant women with heart diseases					

ECHOCARDIOGRAM COMPETENCIES

Echo	Basic Echocardiography competencies	Date 1	Date 2	Date 3	Date 4	Date 5
#	Number of echocardiograms done with supervision of MT/mentor/Specialist in the days of mentorship					
#	Number of echocardiograms with a correct final diagnosis supervised by MT/Mentor/Specialist in the days of mentorship					
E1	Demonstrates an understanding of basic ultrasound principles					
E2	Understands the probe care document and implements it correctly					
E3	Demonstrates knowledge about key ultrasound adjustments to optimize image quality (axis, depth and gain)					
E4	Shows correct technique to perform the 5 basic views in cardiac ultrasound (PLAX, PSAX, A4C, Subcostal, IVC) in children					
E5	Shows correct technique to perform the 5 basic views in cardiac ultrasound (PLAX, PSAX, A4C, Subcostal, IVC) in adults					
E6	Describes the echo features of cardiomyopathy, mitral stenosis, hypertensive heart diseases, right heart failure, pericardial diseases					

E7	Describes the echo features for the most common congenital heart diseases					
E8	Correctly identifies diagnosis according to the PEN-Plus protocol					
E10	Able to clearly and correctly report echo results					

SICKLE CELL DISEASE

	Clinical Competencies	**Date 1**	**Date 2**	**Date 3**	**Date 4**	**Date 5**
	DIAGNOSE AND ASSES SCD					
S1	Proactively uses the diagnostic algorithm in the country/PEN-Plus facility for people with signs and symptoms of SCD					
S2	Accurately interprets results of SCD rapid tests and/or Hb Electrophoresis					
S3	Differentiates sickle cell trait from sickle cell diseases based on diagnostic test results and clinical context					
S4	Performs a comprehensive baseline assessment of a newly diagnosed SCD patient including history, physical examination and appropriate laboratory investigations.					
	MANAGEMENT OF SCD					
S5	Initiates hydroxyurea at the correct starting dose based on patient weight and baseline laboratory evaluation (FBC/Hb)					
S6	Adjusts HU dosing appropriately according to updated patient weight and clinical response and laboratory monitoring.					
S7	Monitors correctly for hydroxyurea toxicity using full blood count or isolated Hb (if FBC not available), kidney and liver function when indicated					
S8	Recognizes clinical and laboratory signs of HU toxicity.					
S9	Temporary stops hydroxyurea when toxicity occurs					
S10	Re-starts hydroxyurea correctly when toxicity is solved					
S11	Prescribes folic acid supplementation to reduce complications related to chronic hemolysis.					
S12	Assess immunization status in children using the under-5 card and recommends indicated vaccines according to national guidelines.					
S13	Uses antibiotic and or malaria prophylaxis when indicated based on National Policy (usually for under 5)					

	Clinical Competencies	**Date 1**	**Date 2**	**Date 3**	**Date 4**	**Date 5**
S14	Provides routine follow-up care including monitoring of growth, complications, treatment adherence and medication tolerance					
S15	Recommends temporarily stopping HU during the first trimester of pregnancy					
S16	Re-starts HU during the second trimester if pain crisis, need of transfusion and/or other complications in agreement with patient					
	MANAGEMENT OF ACUTE AND CHRONIC COMPLICATIONS					
S17	Identifies Vaso - occlusive crisis and determines when hospitalization is required					
S18	Identifies suspected acute chest syndrome and refers for hospitalization					
S19	Recognizes signs of stroke and refers for hospitalization					
S20	Suspects acute splenic sequestration or severe acute hemolytic anemia and refers for hospitalization					
S21	Identifies severe anemia requiring urgent blood transfusion and triggers					
S22	Identifies indications for non-urgent blood transfusion and applies appropriate hemoglobin thresholds.					
S23	Identifies signs of infection/sepsis, checks for malaria, starts empiric broad spectrum antibiotics based on immunization status and refers for hospitalization when necessary					
	PAIN MANAGEMENT					
S24	Evaluates severity of pain using appropriate tools					
S25	Identifies acute Vaso-occlusive pain crisis					
S26	Initiates pain management using the WHO analgesic ladder					
S27	Prescribes morphine at age-appropriate dosing when indicated/available					
S28	Demonstrates ability to titrate up and down morphine for pain and knowledge of side effects					
S29	Identify when acute pain management requires hospitalization. Evaluates patients for other potential causes of pain or comorbid conditions					
S30	Able to differentiate acute from chronic pain					
S31	Identifies patients with AVN as cause of chronic pain					

	Clinical Competencies	**Date 1**	**Date 2**	**Date 3**	**Date 4**	**Date 5**
S32	Utilizes adjunct medication in chronic pain management and nonpharmacological approaches to pain					
S33	Refers to physical therapy if available and appropriate					
	INPATIENT MANAGEMENT					
S34	Safely transitions between oral and intravenous opioid therapy during hospitalization.					
S35	Manages acute chest syndrome safely including blood transfusion					
S36	Manages Stroke safely including blood transfusion					
S37	Manages fever in inpatients with appropriate evaluation and antimicrobial treatment					
S38	Manages other complications such as priapism, adequately					
	PATIENT EDUCATION					
S39	Provides counselling to parents and patient related to acute and chronic complications (especially management of pain), early warning signs and when to come to the hospital					
S40	Recognizes the role and types of psychosocial support needed in SCD management					
	MANAGEMENT OF SCD DURING AND BEFORE PREGNANCY					
S41	Refers patients of childbearing age for family planning as needed and discusses risks of pregnancy in SCD					
S42	Re-starts HU during the second trimester if pain crisis, need of transfusion and/or other complications in agreement with patient					
S43	Initiates preventive therapies during pregnancy: aspirin 150 mg after week 12 and calcium 1.5 gr daily					
S44	Ensures appropriate ANC follow up schedule (every 4 weeks until week 28, every 2 weeks until week 36 and weekly after week 36)					
S45	Starts low molecular weight heparin (LMWH) in inpatient pregnant women or outpatient with history of thrombosis or known clotting disorder					

RESPIRATORY DISEASES

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
	ASTHMA: ASSESSMENT AND DIAGNOSIS					
R1	Performs a structured clinical assessment for Asthma including symptom history (wheeze, cough, nocturnal symptoms, triggers) and physical examination.					
R2	Recognizes typical patterns of asthma symptoms, including reversibility and flares.					
R3	Differentiates asthma from other causes of chronic respiratory symptoms such as COPD, post-tuberculosis lung diseases, heart failure or infection					
R4	Understands and performs spirometry (if available) as a diagnostic tool and differentiates between obstruction and restriction.					
R5	Assess and classifies asthma using GINA or local guideline symptom control tools.					
R6	Identifies correctly asthma as controlled, partially controlled or uncontrolled.					
R7	Identifies risk factors for severe asthma exacerbation (poor adherence, frequent use of reliever, environmental exposure)					
	MANAGEMENT OF ASTHMA					
R8	Prescribes inhaled steroid as the foundation of treatment for all patients with persistent asthma when available.					
R9	Prescribes and adjusts inhaled therapies according to asthma severity and control using stepwise treatment (step up and step down)					
R10	Avoids use of LABA (long-acting beta agonist) as monotherapy in asthma.					
R11	Avoids routine use of salbutamol and theophylline tablets in Asthma due to low effectiveness and higher toxicity					
R12	Prescribes oral corticosteroids appropriately during acute exacerbations.					

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
R13	Prescribes low oral doses of corticosteroids only if no other drugs available for symptomatic control out of exacerbations					
	PATIENT EDUCATION					
R14	Counsels patients on identifying and avoiding common asthma triggers (dust, smoke, cold air,)					
R15	Counsels on stopping smoking					
R16	Demonstrates inhaler technique to patient and verifies patient ability to perform it correctly (including spacer)					
R17	Teach patients how to adjust treatment according to symptom control and how to monitor symptoms and recognize poor asthma control and confirm back understanding.					
	MANAGEMENT OF ASTHMA EXACERBATIONS					
R18	Recognizes signs of asthma exacerbation including increased asthma exacerbation including breathlessness, wheezing, reduced peak flow, or inability to speak in full sentences.					
R19	Identifies danger signs requiring urgent referral or hospitalization (severe respiratory distress, hypoxia, altered consciousness,)					
R20	Initiates appropriate treatment of mild or moderate exacerbations in outpatient settings and monitors patient response to treatment and determines when escalation of care or referral is required					
	ASSESSMENT, DIAGNOSIS OF OTHER CHRONIC RESPIRATORY DISEASES (COPD AND POST-TUBERCULOSIS LUNG DISEASE)					
R21	Evaluates patients with chronic respiratory symptoms using structured history including smoking, tb, occupational exposures)					
R22	Recognizes clinical features of COPD including chronic cough, sputum production, and progressive dyspnea.					
R23	Recognizes common post TB patterns in the Xray					
R24	Differentiates obstructive and restrictive respiratory patterns using spirometry when available					
	MANAGEMENT OF COPD AND POST-LUNG DISEASES					
R25	Identifies exacerbation of COPD and when to refer for admission					
R26	Correctly manages exacerbation when there is no indication for hospitalization (bronchodilators, steroids and antibiotics when indicated)					

SEVERE HYPERTENSION

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
	ASSESSMENT AND DIAGNOSIS					
H1	Measures BP correctly several times using an appropriate technique before giving a diagnosis of elevated blood pressure.					
H2	Performs a focused clinical evaluation to identify target organ damage with the appropriate baseline investigations: kidney function, electrolyte, blood glucose, and other tests according to guidelines.					
H3	Assesses cardiovascular risk factors including Diabetes, obesity, smoking and family history.					
	MANAGEMENT					
H4	Starts medication adequately (first line drugs at adequate dosage) long acting CCB, diuretics or ACEI based on hypertension stage and comorbidities.					
H5	Initiates combination therapy when indicated (stage 2 or severe hypertension)					
H6	Titrates up medication when BP targets are not achieved. If the BP is uncontrolled with medium doses of 2 drugs, adds third drug (most effective option) or increases the doses to maximum ones based on adherence or availability					
H7	Optimizes treatment regimen before labeling hypertension as resistant					
H8	Initiates spironolactone as four drugs for resistant hypertension (unless hyperkaliemia)					
H9	Uses beta-blockers as additional therapy when needed for resistant hypertension or when indicated by comorbid conditions					
H10	Maintains antihypertensive therapy when blood pressure targets are achieved unless adverse effects occur.					
	MONITORING AND SAFETY					
H11	Checks for kidney function before and after starting and adjusting ACEI if available.					

	Clinical Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
H12	Considers stopping or consulting on the continuation of ACEI if creatinine levels increase > 30% after initiation from the baseline					
H13	Identifies important adverse effects of drugs (e.g. angioedema and cough with ACE inhibitors, edema with amlodipine)					
	EMERGENCY AND URGENT CARE					
H14	Can differentiate between hypertensive urgency and emergency.					
H15	Recognizes clinical signs of acute organ damage (stroke, acute heart failure, chest pain, kidney failure, altered mental status) requiring admission					
H16	Manages hypertensive urgency correctly at OPD with oral medications and close follow-up.					
H17	Appropriately manages hypertensive emergency					
	PATIENT EDUCATION					
H18	Provides counselling on lifestyle interventions including salt reduction, weight control, smoking cessation and physical activity					
H19	Educates patients about hypertension as a chronic condition requiring long-term treatment and monitoring, and the importance of medication adherence					

CHRONIC KIDNEY DISEASE AND NEPHROTIC SYNDROME

	Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
	ASSESSMENT AND DIAGNOSIS OF CKD					
H1	Identifies when a patient has chronic kidney failure based on lab markers of kidney failure (Creatinine, and glomerular filtration)					
H2	Performs essential first-visit evaluations for patients with CKD including Blood pressure, Urine dipstick, HIV, kidney function, electrolytes and abdominal USS (if available)					
H3	Identifies the common causes of CKD in the local context: HTN, DM, Obstructive uropathy, HIV associated kidney failure, others,					
H4	Checks for symptoms and signs of urine retention in Male patients > 50 years/or with symptoms and signs of benign prostatic hypertrophy					
	MANAGEMENT OF CKD					
H5	Treats BP with a goal of blood pressure of less than 130/80 mmHg, starting with low-dose ACE inhibitor if proteinuria and any other antihypertensive medication if proteinuria is absent.					
H6	Checks creatinine increases after starting ACE inhibitors and adjusts when the increase is > 30% from the baseline levels or potassium levels are > 5.5 mEq/L after consulting an expert					
H7	Avoid the use of ACE inhibitors in stage 4 or 5 unless prescribed by Nephrologist after clinical evaluation.					
H8	Identifies and avoids nephrotoxic medications as non-steroidal anti-inflammatory drugs (NSAIDs), gentamycin, tenofovir and other drugs toxic to kidney					
H9	Manages volume overload using furosemide and/or thiazide in any of the stages.					
H10	Screens and treat anemia associated with CKD with iron supplementation if Hb < 10 g/dl					
H11	Identifies the main complications of CKD (hyperpotassemia, volume overload, encephalopathy) and refer for admission and urgent dialysis when needed					
H12	Monitors progression of CKD through clinical assessment and laboratory data					

	Competencies	Date 1	Date 2	Date 3	Date 4	Date 5
	COORDINATION OF CARE AND REFERRAL					
H13	Refers to **all patients with CKD** patients for nephrology evaluation when specialized care is available.					
H14	Refers for family planning to women in childbearing age					
H15	Identifies patients with advanced CKD who might benefit from palliative care when dialysis and transplant is not feasible					
	ASSESSMENT AND DIAGNOSIS OF NEPHROTIC SYNDROME					
H16	Identifies signs and symptoms of nephrotic syndrome (generalized edema, foamy urine, periorbital swelling,)					
H17	Identifies common causes of nephrotic syndrome by age group (minimal change diseases, post-infectious, HIV associated,)					
H18	Use urine dipstick testing to detect significant proteinuria as an initial diagnostic tool.					
H19	Differentiates nephrotic syndrome from other causes of edema such as heart failure or liver disease					
	MANAGEMENT OF NEPHROTIC SYNDROME					
H20	Initiates corticosteroid therapy at appropriate dosing for suspected steroid-sensitive nephrotic syndrome.					
H21	Gradually tappers down corticosteroids when remission is achieved.					
H22	Identifies steroid resistance when proteinuria does not improve after adequate course of therapy and refers to specialist					
H23	Initiates ACE inhibitors as adjunct therapy to reduce proteinuria when appropriate.					
H24	Manages edema using diuretics as furosemide and spironolactone.					
H25	Adds statins (if available) when indicated (8 years, persistent proteinuria and high cholesterol)					

EPILEPSY

	EPILEPSY COMPETENCIES	**Day 1**	**Day 2**	**Day 3**	**Day 4**	**Day 5**
	ASSESSMENT AND DIAGNOSIS					
E1	Conducts a structured seizure history (onset, duration, frequency, triggers, pattern,)					
E2	Screens for potential precipitants of crisis if needed: sleep deprivation, missed doses, alcohol, ...					
E3	Assesses adherence, side effects and barriers to treatment if needed (cost, distance, superstition, stigma,)					
E4	Classifies seizures into generalized vs focal based on clinical history					
E5	Differentiates epileptic seizures from common mimics such as syncope, febrile seizures, psychogenic non-epileptic events.					
E6	Performs a focused neurological examination to identify focal neurological deficits or other signs suggesting secondary causes					
E7	Recognizes when to refer for the performance of secondary tests such as CT or EEG					
E8	Identifies when to refer to higher level care: recurrent seizures despite 2 optimized drugs, suspicion of structural cause (focal neurological signs or persistent headache)					
	MANAGEMENT					
E9	Initiates appropriate first line antiseizure medications available locally or based on local protocol at adequate doses.					
E10	Follows stepwise protocol in which drug to use. Uses a max dose of one drug before switching to another.					
E11	Titrates dose of anti-convulsing according to level of control and side effects					
E12	Changes drugs in case there is intolerance or lack of response					
E13	Considers interactions with anticonvulsants and other drugs (Tuberculosis treatment, Antiretroviral treatment or contraceptive)					
	PATIENT EDUCATION					
E15	Addresses stigma and misconceptions about epilepsy in the community and family.					

	EPILEPSY COMPETENCIES	**Day 1**	**Day 2**	**Day 3**	**Day 4**	**Day 5**
	ASSESSMENT AND DIAGNOSIS					
E16	Counsels patients on adherence to medication and avoidance of known seizure triggers and to keep a seizures diary					
	PREGNANCY AND EPILEPSY					
E17	Provides counselling to women of childbearing age regarding pregnancy planning and epilepsy management					
E18	Avoids sodium valproate in women of childbearing when other alternatives are available.					
E19	Provides counselling about contraception and refers to the contraception units when necessary					

PALLIATIVE CARE

	PALLIATIVE CARE COMPETENCIES	**DAY1**	**DAY2**	**DAY3**	**DAY4**	**DAY5**
	Assessment of needs and symptoms					
P1	Identifies palliative care needs of clients and their family/care giver					
P2	Provides standard pain assessment (application of concept TOTAL Pain) and assessment of other symptoms					
	Pain and symptom management					
P3	Uses standard pain management (using WHO analgesic ladder)					
P4	Understands the use of adjuvants for the management of the pain and secondary effects					
P5	Knows how to apply symptoms control for common symptoms (including non-pharmacological approaches)					
P6	Knows the side effects and drug-drug interaction of common symptom control medication (top priority for side effects of opioids and how to prevent these side effects)					
P7	Limits the use of drugs that are not indicated in a situation of palliative care					
	Psychosocial, cultural and spiritual support					
P8	The provider assesses psychosocial needs, provides social support, and refers to a mental health specialist.					

	PALLIATIVE CARE COMPETENCIES	DAY1	DAY2	DAY3	DAY4	DAY5
P8	Knows how to provide or when and where to refer for spiritual and cultural care					
P10	Understands the special considerations needed for pediatric clients					
P11	Limits the use of drugs that are not indicated in a situation of palliative care					
	Care Coordination & End of life Planning					
P12	Plans for end-of-life care when indicated					
P13	Links the patient to any home-based care or community resources					

CHRONIC LIVER DISEASE

	Clinical Competencies	Day 1	Day 2	Day 3	Day 4	Day 5
	ASSESSMENT AND DIAGNOSIS					
L1	Knows and uses noninvasive scores to diagnose liver cirrhosis (APRI, USS), with adequate interpretation of abdominal USS findings					
L2	Identifies etiologies of liver disease (eg, viral hepatitis, alcoholic hepatitis, steatosis liver disease) in clinical history and complementary tests					
L3	Performs targeted physical exam looking for: jaundice, ascites, pedal edema, spiders and asterixis to identify complications					
L4	Identifies signs of overload decompensation (ascites)					
L5	Identifies signs of Liver encephalopathy and its triggers					
L6	Identifies signs of GI bleeding and performs rectal exam when suspected					
	GENERAL MANAGEMENT AND OF COMPLICATIONS					
L7	Initiates specific treatment for underline treatable conditions as hepatitis B and C					
L8	Initiates diuretics correctly when ascites or pedal edema.					
L9	Performs paracentesis when indicated (ascites refractory to diuretics)					
L10	Titrates diuretics correctly up and down with electrolyte and kidney function monitoring.					

L11	Treats correctly initial signs of liver encephalopathy if there is no indication for admission with lactulose, and identification of triggers.					
L12	Identifies when to refer for admission when the patient shows signs of decompensation					
L13	Gives Beta blockers for patients with previous esophageal varices bleeding					
L14	Titrates betablockers correctly to achieve benefit (lowering HR to at least 55 bpm)					
L15	Gives antibiotics for secondary prophylaxis of spontaneous bacterial peritonitis					

FOLLOW-UP FOR MENTORS OR MASTER TRAINERS

GAP mapping grid

Date of GAP identification	Define GAP	Domain GAP (knowledge, critical reasoning, clinical skills, communication, attitude)	Cover during mentorship (yes/no) or addressing later?	Timeline to address it

What level of supervision does this provider require for this condition after five mentorship sessions?

- ❑ Intensive mentorship
- ❑ On going mentorship
- ❑ Independent practice